

METHODS for EVALUATING DIGITAL HEALTH INTERVENTIONS

Digital health interventions projects:

- explore basic questions as to whether intervention addresses identified needs

including $\leftarrow$ technical functionality
feasibility

- assessment of user satisfaction

- evaluate effectiveness attributable impact

- value for money

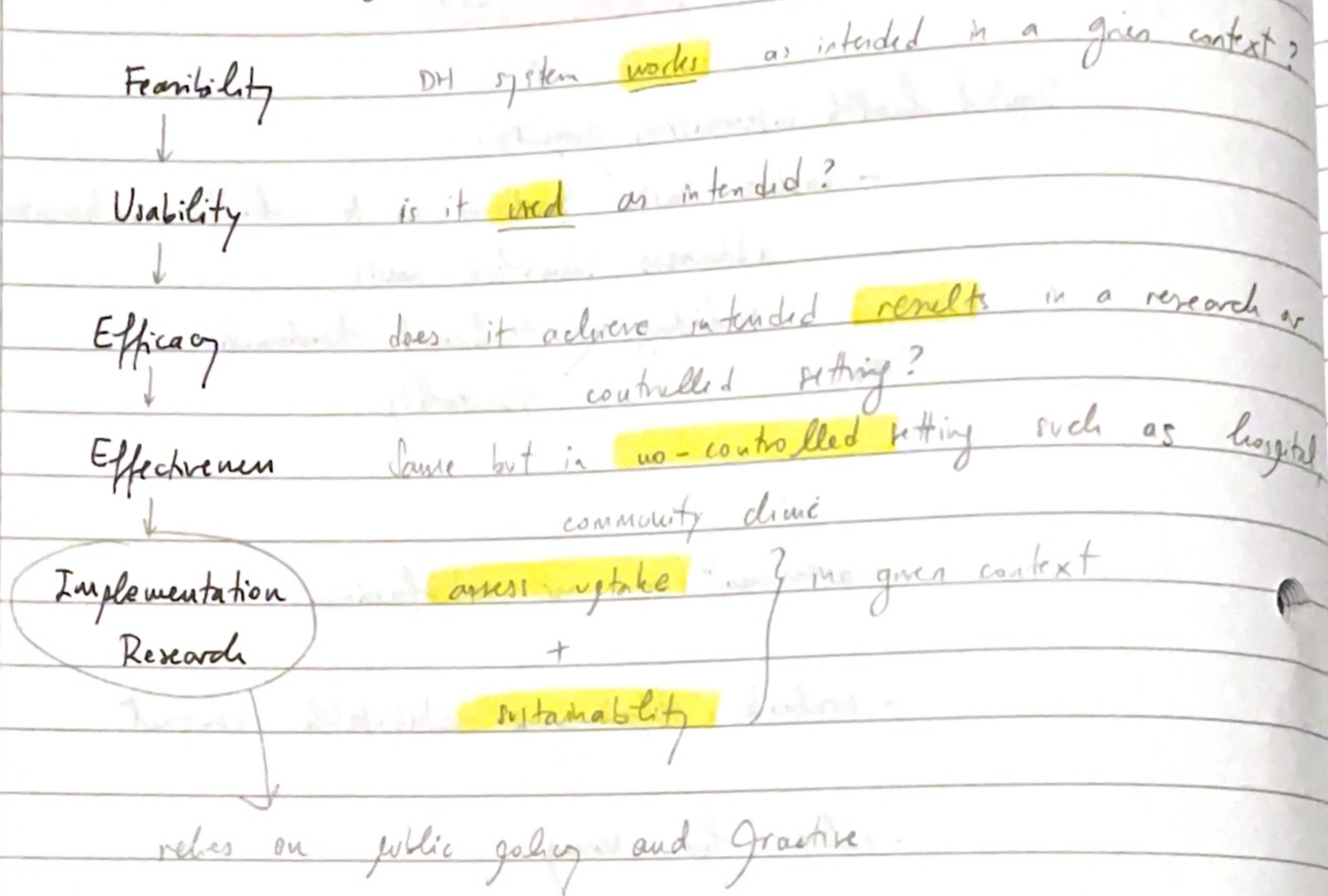
Monitoring
Continuous process $\leftarrow$ collecting + $\rightarrow$ DATA $\rightarrow$ compare results vs. expectations
analysis

in DH, Green monitoring $\leftarrow$ routine collection
review
analysis of data
measures
implementation

WHO $\rightarrow$ Six stages of intervention maturity life cycle

- 1) preprototype
- 2) prototype
- 3) pilot
- 4) demonstration
- 5) scale up
- 6) integration / sustainability

Stages to evaluating DH interventions:



read

chapter 4
WHO

Evaluating
relevant re

estab
the c
estab

RCT

Quasi-

Observat

Descript

Evaluation Strategy
relevant research questions include → define problem
study benefit

→ establish the likely reach and uptake of the intervention
the causal model describing how the intervention ^{will reach} → intended benefit
estimate overall benefit → effectiveness
costs
harms

RCT → gold standard

only when : → intervention + delivery package are stable
implemented with high fidelity
reasonable likelihood of clinical effectiveness

Quasi-experimental (non randomised)

aim to → demonstrate causality
not randomised → can be biased

→ interrupted time series methods

Observational studies → cannot demonstrate causation

Descriptive → who / when / where

2 main types → individuals
population

may include qualitative

RCTs - any $\neq$ in results (group A, group B) is attributed to the intervention

1) $\rightarrow$ VALIDITY

Internal validity

Randomisation

Allocation concealment

$\rightarrow$ prevents bias
at the very moment of assignment

Statistical power $\rightarrow$ ability to detect a $\neq$

determined by $\left\{ \begin{array}{l} \text{frequency of outcome being studied} \\ \text{magnitude of effect} \\ \text{study design} \\ \text{sample size} \end{array} \right.$

2) $\rightarrow$ RELIABILITY

1) how big is the treatment effect

2) how likely results are $\neq$ chance alone

Statistical parameters

ICC - intraclass correlation coefficient:

$\rho(AHO)$

relatedness of clustered data by comparing

variance
within clusters vs between clusters

$\rightarrow$ 95% confidence intervals

$\rightarrow$ magnitude of effect measure

$\rightarrow$ p values

$\rightarrow$ Also consider clinical significance

value of results for an individual patient

Δ effect size (group A - group B)

$\downarrow$

important to make a decision

(regardless of statistical significance)

3) Precision around estimate of intervention effects

95% → how precise → wider the interval → ↓ precision

4) Using and appraising RCT

check

types

ADAPTIVE DESIGNS

informative
⊕ efficient
ethical

⊖ more complex

designs → RCT more flexible → obtaining results → modify trial's course in accordance with pre-specified rules

NATURAL EXPERIMENTS

↑ bias → use ≠ methods
testing
sensitivity analyses } as additional checks before making causal inferences

+ explicitly recognize assumptions

TO ↑
Validity

Babylon example → to read

NICE → Evidence Standards framework for DH technologies
evidence should be
available or developed for
not so useful for < DH private
adaptive algorithm

effectiveness
economic impact

ECONOMIC EVALUATION

1) -

2 parameters → cost
outcome

→ essential features → cost + outcomes are analyzed
more than 1 strategy must be compared

5 main economic evaluation methodologies

focus of	• Cost effectiveness analysis	technical efficiency
	• " utility	" "Quality of life"
	• " benefit	• economic efficiency
	• " cost-effectiveness	" all direct / costs indirect
	• " minimisation	• only inputs

measurement of outcome determines which

see again
last
video

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